

SECTION IV

Treatment

THE THERAPIST ROLE: CONNECTION AND COLLABORATION

Our radical assumption in IFS is that everybody has an undamaged, strong, clear, calm essence and the capacity to function with agency. As a result, we have neither the right nor the authority to take any protection away from the client, including their methods of soothing and distracting. In IFS, we don't try to control clients and we don't judge them; we help them connect with their controlling and judgmental managers. We also don't argue with firefighters who are distracting from sadness and fear because doing so would amount to more distraction from the main issue, which is exiled pain.

By the same token, it is not our responsibility to determine the outcome of treatment. We assume clients have lost their access to Self-leadership and self-trust, but we also believe they still have the capacity for self-compassion and self-governance. We help them locate and build on it. To that end, our first and ongoing aim is to create order out of chaos. We welcome the client as they appear now (which is generally stressed out and overwhelmed by internal conflict), and as a way of building Self-to-part connections, we help them identify their parts, understand the roles of these parts, and see how they group together into teams. As the Self takes the lead, clients learn the intentions behind parts' behaviors and become willing to try alternative actions in the future. Like most therapy, this takes time.

The deeper the client's pain and disorganization, the longer and more intense the therapy—and the more pressure on the therapist's system. Addictive processes are uniquely stressful for the manager parts of therapists, who are prone to judging, caretaking, and panicking. The client's anxiety, turmoil, and risk-taking exert a centrifugal draw on the resources of everyone in their vicinity, especially therapists who have responsible managers. For these parts, it can take more energy to resist a bid for help than it takes to dive in with directives. But giving in to that urge will prove highly ineffective. Short of an acute crisis in which intervention is necessary, taking the bulk of decision-making out of the client's hands on a regular basis (which happens in many treatment centers) is bound to be counterproductive. We can forestall our caretaking, critical, and controlling parts from blending and siding with the client's managers by listening to our own protectors and helping our exiles as needed, with supervision if necessary (Redfern, 2022).

When we feel helpless and dismayed with our clients, we may become blended with our firefighter parts, who may want to escape the client's distress, punish their lies, or steer clear of their anger. Although it is only natural for our firefighters to respond this way when a client is damaging themselves and hurting others, we still must prevent them from overtaking therapy. With this aim, we can quickly check inside, validate our reactive parts, normalize their feelings, and ask them to trust us. To the client, we may speak out loud about our need to press pause and collect our thoughts. If that's not possible, we can be transparent, letting the client know that one of our parts is reacting to one of their parts and naming our part.

Sometimes our exiles will escalate. We see them in panic, fearing incompetence, and full of shame. Although we may choose to speak for certain exiles when we are in session with a client—for example, naming them in the course of apologizing for a judgmental protector—we generally help our vulnerable parts outside of session. When we routinely make time for them, they are more willing to stay separate and wait for a bit when they need help.

That said, working closely with a system that is distressed and unbalanced can activate exiles and protectors in even the most level-headed clinician. When our protectors feel the need to activate, we go to our vulnerable parts. How have they been affected? What do they need to feel reassured right now? Whether or not we can ever unburden all of our exiles, we need to have the experience of feeling open-hearted compassion, negotiating with our protectors, witnessing our exiles, being with them as they unburden, and feeling their relief. This is how we gain the courage to befriend the firefighters of our clients and listen to their good intentions.

Any kind of parts takeover in the therapist that is not acknowledged, even if it's brief, will have a negative effect. Many clients have vigilant parts detectors and can easily perceive the therapist's parts. We become accountable by acknowledging our role in any relationship rupture, welcoming the client's feedback, and inviting them to tell us how it felt to be on receiving end of our behavior. By speaking for our parts and promising to care for them, we make a repair and preserve the safety of the therapeutic relationship.

We also model practicing what we preach. The client's protectors need to know that the therapist is on their side. Apologies can be a powerful intervention in this regard. There is no shame in us or our clients having protectors and exiles. As the Buddhist teacher Pema Chödrön (2007) wrote, "Compassion is not a relationship between the healer and the wounded. It's a relationship between equals. Only when we know our own darkness well can we be present with the darkness of others. Compassion becomes real when we recognize our shared humanity" (p. 50).

COMMON THERAPIST MANAGERS

- **Controlling:** *You have to get rid of it! (food, drugs, phone numbers, and so on)*
- **Caretaking:** *I can call that agency for you.*
- **Judging:** *You should know better than that by now.*
- **Critical:** *Wow. That was a big mistake! You really blew it this time.*
- **Overly responsible:** *It's my fault this client isn't getting better. I should do more.*
- **Perfectionistic:** *That was okay, but this would have been better.*

COMMON THERAPIST FIREFIGHTERS

- **Angry:** *This client doesn't even listen to me! They waste my time.*
- **Distracting:** *Let's just focus on your work situation today.*
- **Colluding:** *Yeah, it's not really that big a deal.*
- **Escapist:** *I'm sorry, but I have to cancel our appointment.*
- **Soothing:** *Tonight, I deserve _____ (to have a few drinks, get high, eat ice cream, watch TV, zone out on social media, and so on).*

- **Dissociating:** *Can you repeat that?*

COMMON THERAPIST EXILES

- **Hopeless:** *This client is never going to change.*
- **Ashamed:** *I'm not good enough. I never get it right!*
- **Guilty:** *I am a terrible person. I'm not empathic enough.*
- **Overidentified:** *You're just like me.*
- **Overwhelmed:** *I don't know where to start.*
- **Sad:** *This reminds me of my childhood—it's too sad!*

Therapist Exercise

Get to Know Three Sets of Parts

Therapists who are learning the IFS approach to addictive processes will encounter three sets of parts who have different interests and needs. In this exercise, we invite you to notice and reach out to each set of parts. You can do all three sections at once, but if that feels challenging, feel free to go more slowly and do each section at a different time. Read through to the end of this exercise before making a decision on how to proceed. If you think you would benefit from having someone to talk with, ask a colleague to do the exercise with you or to be available for a check-in once you've finished.

Here are the three sets of parts:

- Therapist parts who aim to learn new interventions and get new ideas for reaching your clients.
- Parts who focus on your family members or other loved ones who suffer from various addictive processes.
- Parts who focus internally, on your own addictive processes, whether in the past or currently. (If you can't think of a personal struggle that rises to the level of an addictive process, we invite you to think of any activity that causes some measure of internal conflict. For example, perhaps one part likes a certain activity but another part wants you to stop.)

Get ready to inquire into these three sets of parts. First, take time to find privacy, get into a comfortable seated position, and relax. If it's okay with you, close your eyes and take a few deep breaths. Breathe fully and deeply, stretching your inhalation all the way down your back and feeling the chair or floor that supports you. Then breathe out slowly. Repeat a few times.

Next, welcome your therapist parts—the ones who want new ideas for your clients. What do they want to learn about the addictive process? What are their strengths? What are their challenges? Are they preoccupied with any particular clients today? Do they hold any clients with addictive processes, even from the past, in your heart? Do they react to or find it hard to connect with any particular clients? Listen to these parts. When you're ready, write down their answers. How do you feel toward them now? Tell them you've been listening. Thank them for whatever they shared. Let them know they can stay in relationship with you.

Now, turn to any parts who focus on family members or loved ones who have struggled with addictive processes in the past or who struggle now. Notice the images or memories they hold. If you'd like, write down the name of each family member or loved one. What do your parts want you to know about this person? How have they been affected by this person? Has your connection with this person changed at all (that is, gotten better or worse)? If so, what happened?

Take time to record what your parts are holding for each of the people mentioned. How are these parts doing now? Listen carefully. Do they notice you? What do they want you to learn from this manual? Thank them for whatever they shared. Let them know they can stay in relationship with you.

Finally, when you are ready, invite any parts who promote some type of addictive process to serve you. What do you notice about them? How do you feel toward them? Do you notice any other parts reacting toward them? If so, ask the reactive ones to step back and trust you. How have they tried to help? What do the addictive parts need from you today? What do they want you to understand or learn from this manual? Take time to record their answers and thank them for whatever they shared. Let them know they can stay in relationship with you.

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OUR AGENDA

In IFS, we don't come at protectors in an additive system with our own change agenda. This stance may feel counterintuitive or even downright unethical for a couple of reasons. First, people whose addictive parts cause dysfunction for themselves are bound to annoy and worry both themselves and others. Second, therapists are trained (and thereafter continually urged) to pursue change in their clients. But with extreme protectors, it's most effective to think systemically, be optimistic, and have no agenda other than getting them connected to the Self and letting them explain their positive intentions. For this to happen, our own managers